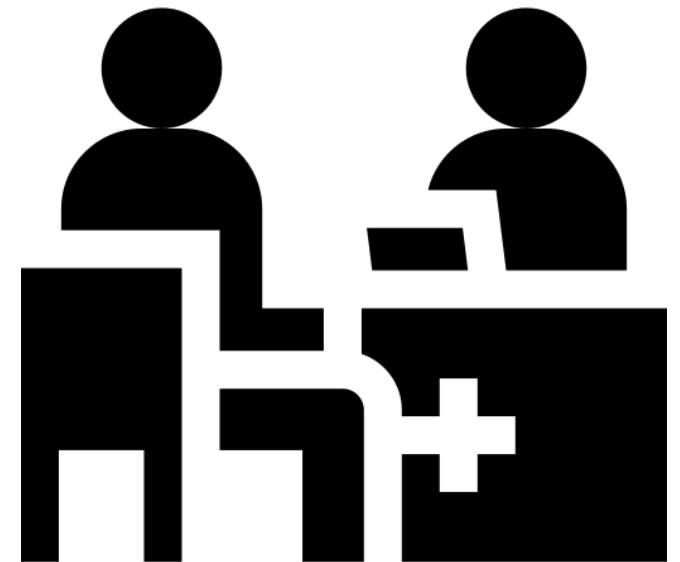


Attentive listening & summarization



Learning objectives



- Define attentive listening and summarization in clinical communication.
- Explain why listening and summarizing are essential for patient care and learning.
- Identify barriers to effective listening and summarization.
- Demonstrate key behaviors/techniques for attentive listening and summarizing patient concerns.
- Reflect on how these skills apply in the clinical context

What is attentive listening?

- Simply hearing $\neq$ listening
- Listening is a conscious, engaged process.
- Verbal and non-verbal attention
- Reflect and clarify patient statements
- it involves actively processing and seeking to understand the meaning and intent behind them
- It is the single most important skill for building trust and gathering accurate data.

Why attentive listening?

- Diagnosis is based on patient's words
- Allows exploration of patient's concern
- Improves outcome
- Reduces error
- Builds trust
- Helps in summarization, planning, and decision making

Components of Attentive Listening (Non-verbals)

- Presence: be mentally and physically present
- Facilitation: Nodding, using gestures, and making encouraging utterances ("Go on," "I see," "Uh-huh").
- Body Language: Leaning slightly forward, maintaining an open posture.
- Eye Contact: Maintaining appropriate, culturally sensitive eye contact.
- Environment: Minimizing distractions (e.g., facing the patient instead of the computer screen).

Components of Attentive Listening (Verbals)

- **Allowing Silence:** Giving the patient time to think and respond without interruption. Silence is a powerful tool that often yields the most important information.
- **Not Interrupting:** Letting the patient complete their thought before you speak. Studies show doctors interrupt patients within 11-18 seconds. Fight this impulse.
- **Invitations to Talk:** "Could you tell me more about that?"

S

Sitting at a comfortable angle and distance.

O

Open posture.
Arms and legs uncrossed.

L

Learning forward from time to time.
Looking genuinely interested.
Listening attentively.

E

Effective eye contact without staring.

R

Remaining relatively relaxed.

Some non-verbal cues

- Facial expressions
- Gestures
- Nodding
- Leaning forward

Some verbal cues

- “I see...” / “Go on...”
- Paraphrasing statements
- Asking clarifying questions

Common mistakes

- Interrupting
- Judging prematurely
- Thinking ahead instead of focusing
- Multitasking

Key notes of attentive listening

- Active engagement is key
- Observe verbal & non-verbal cues
- Practice reflective and paraphrasing skills

What is summarization?

- Condensing key patient information
- Repeating/rephrasing the condensed information
- Reflects understanding
- Confirms alignment between clinician and patient

Why summarization matters?

- Confirms accurate understanding
- Ensures you have not missed vital information
- A chance for the patient to add on information
- Builds trust with patients
- Encourages patient engagement
- Prevents errors in diagnosis and treatment

When to summarize?

- After patient explains symptoms
- During transitions in the conversation
- When patient expresses emotions or concerns
- At the end of the interview

Steps of summarization

- Listen fully
- Identify key points
- Rephrase clearly

Confirm understanding:

Did I say everything correctly?

Is there something which I might have missed?

Is there anything else to add?

Steps of summarization

1. Signpost: Announce you are going to summarize. ("So, let me just make sure I have everything correct.")
2. Synthesize: Pull together the key points from the history in a clear, concise manner.
3. Use the Patient's Words: Where possible, this reinforces that you have heard them.
4. Invite Correction: End the summary by asking for input. ("Does that sound right?" or "Is there anything I've missed?")

Common mistakes

- Interrupting the patient to summarize
- Adding your own interpretation
- Summarizing too briefly or vaguely

Common barriers

- Distractions
- Time pressure
- Fatigue
- Personal biases

Overcoming barriers

- Minimize distractions
- Allocate sufficient time per patient
- Practice mindfulness to reduce fatigue
- Recognize and bracket personal biases

Case Scenario 1

- Scenario 2: Focusing on Attentive Listening
- Title: The Dismissed Concern Patient: Ms. Khadija, a 19-year-old university student. Opening Line from Patient: "I know you're probably going to think this is silly, but my stomach has been a bit off." She sighs and looks down at her hands.
- Background: I have had intermittent abdominal cramps and diarrhea for 6 weeks. It started during final exams. I'm stressed about my grades and also about being away from home. I saw a doctor at university clinic 3 weeks ago who said, "It's just stress, drink more water," and made me feel like I was wasting their time. I'm here now because it's not getting better, and worried there's actually something wrong, like Crohn's disease, which my cousin has it. I'm apprehensive and expect to be dismissed again.

Case Scenario 1

- Skill Focus:
- Attentive Listening: The physician must use non-verbal cues (leaning in, nodding) and verbal facilitators ("I'm listening," "Please go on") to encourage the patient. Crucially, they must not interrupt.
- Building Rapport: The physician must work to build trust and a safe environment. This involves acknowledging the patient's initial statement ("Thank you for telling me that. Nothing you feel is silly here"), showing empathy, and demonstrating that they are taking the concern seriously.

Case Scenario 2

- Scenario 1: Focusing on Questioning Techniques & Summarization
- Title: The Vague Symptom Patient: Mr. Ahmad, a 68-year-old retired teacher. He is well-groomed but appears slightly anxious. Opening Line from Patient: "Doctor, I just haven't been feeling like myself lately."
- Background: I have been feeling unusually tired for about 3 months. It's a deep fatigue, not just sleepiness. I also feel a bit lightheaded sometimes, especially when I stand up quickly. I have been gardening less, which I used to love, because I just don't have the energy. I'm worried it might be "something serious," like my heart or cancer, but I don't want to say that outright.

Case Scenario 2

- Skill Focus:
- Questioning Techniques: The physician must resist jumping to closed questions immediately. They should start with open-ended questions ("Can you tell me more about 'not feeling like yourself'?") and then funnel down to focused questions to characterize the fatigue, lightheadedness, and functional impact.
- Summarization: Partway through, the physician should attempt to summarize the story to ensure accuracy before moving on to systems review or past medical history.

Thank you